CONFIDENTIAL Fax

FROM:

JASON THOMPSON, MD
5917 CROSSTOWN EXPY, CORPUS CHRISTI, TX 78417-3504
Phone: (361) 854-0811
Fax: (361) 806-5040

TO:

PAM HEALTH AT HOME
825 MAIN ST, BUDA, TX 78610
Phone: (512) 295-7000
Fax: (512) 295-7070

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PAM HEALTH AT HOME

Fax

To: THOMPSON, JASON	**From:** Pam Health at Home - Texas
Fax: 13618065040	**Pages:** 8
Phone:	**Date:** 03-02-2026 5:25 PM
Re: Order(s) for Signature	

Comments:

This fax contains orders for signature.
Please have the physician/clinician noted on this order.
Please sign, date, and return all pages to 1-512-295-7070.
Thank you!

PAM Health at Home – Texas
825 Main Street, Suite 110, Buda, TX 78610
Ph: 512-295-7000; Fax: 512-295-7070

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Order No:485-26275

HOME HEALTH CERTIFICATION AND PLAN OF CARE

1. Patient's HI Claim No.	2. Start Of Care Date	3. Certification Period	4. Medical Record No.	5. Provider No.
H98517011	02/24/2026	02/24/2026 - 04/24/2026	9389	262009456

6. Patient's Name and Address
Rodriguez, Rosemary C (9389)
5701 Domingo Pena
Corpus Christi, TX 78417
Phone: (361) 816-4175

7. Provider's Name, Address and Telephone Number
PAM Health at Home
825 Main Street, Ste 110 Buda, TX
Buda, TX 78610-3273
Phone: (512) 295-7000, Fax: (512) 295-7070

8. Date of Birth: 08/09/1958 **9. Sex:** ☐ M ☑ F

10. Medications: Dose/Frequency/Route (N)ew (C)hanged

Medication	Dose	Frequency	Route	Status
Amlodipine 10 mg tablet [Amount Of/ Dosage Form: 1 Tablet]	10 mg	daily	oral	LS
Atorvastatin 20 mg tablet [Amount Of/ Dosage Form: 1 Tablet]	20 mg	daily	oral	LS
Celebrex 200 mg capsule [Amount Of/ Dosage Form: 1 Capsule]	200 mg	daily	oral	LS
Gabapentin 100 mg capsule [Amount Of/ Dosage Form: 1 Capsule]	100 mg	3 times a day	oral	LS
Losartan 100 mg tablet [Amount Of/ Dosage Form: 1 Tablet]	100 mg	daily	oral	LS
Oxycodone 10 mg tablet [Amount Of/ Dosage Form: 1 Tablet]	10 mg	every 6 hours	oral	LS - PRN/As Needed

11. ICD-9 / 10 CM Principal Diagnosis

Code	Code Description	Date	O/E
Z47.1	Aftercare following joint replacement surgery		

12. ICD-9 / 10 CM Surgical Procedure

Code	Code Description	Date	O/E

13. ICD-9 / 10 CM Other Pertinent Diagnosis

Code	Code Description	Date	O/E
Z96.651	Presence of right artificial knee joint		
E11.9	Type 2 diabetes mellitus without complications		
I10	Essential (primary) hypertension		
E78.5	Hyperlipidemia, unspecified		
Z79.891	Long term (current) use of opiate analgesic		
Z91.81	History of falling		
Z79.1	Long term (current) use of non-steroidal anti-inflammatories (NSAID)		

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14. DME and Supplies:
DME:
Ppe

15. Safety Measures:
911 Emergency / Protocol, 911 for Emergency, COVID PRECAUTIONS, Emergency Plan / Measures, Emergency Plan Developed, Equipment Safety, Fall / Safety Precautions, Hazard-Free Environment, Infection Control Standard Measures and Precautions, Keep Pathways Clear, Medication Safety / Storage, Safe in Movements, Safety in ADLs, Slow Position Change, Standard Precautions/Infection Control, Support During Transfer and Ambulation, Transfer Precautions, Universal Precautions, Use of Assistive Devices, Walker / Cane

16. Nutritional Req.
HEART HEALTHY. ADA

17. Allergies:
Medication Allergies: Penicillins

18.A. Functional Limitations
☑ 1- Amputation ☐ 4- Hearing ☐ 7- Ambulation
☑ 2- Bowel/Bladder (incontinence) ☐ 5- Paralysis ☐ 8- Speech
☐ 3- Contracture ☑ 6- Endurance ☐ 9 - Legally Blind
☐ A- Dyspnea with minimal exertion
☐ B- Other (Specify):

18.B. Activities Permitted
☐ 1- Complete Bedrest ☑ 5- Exercises Prescribed ☐ 9- Cane
☐ 2- Bedrest BRP ☐ 6- Partial Weight Bearing ☐ A- Wheelchair
☐ 3- Up as Tolerated ☐ 7- Independent at Home ☑ B- Walker
☐ 4- Transfer Bed/Chair ☐ 8- Crutches ☐ C- No Restrictions
☐ D- Other (Specify):

19. Mental Status
☑ 1 - Oriented ☐ 2 - Comatose ☑ 3 - Forgetful ☐ 4 - Depressed ☐ 5 - Disoriented ☐ 6 - Lethargic ☐ 7 - Agitated
☐ 8 - Other(Specify):

20. Prognosis: ☐ 1- Poor ☐ 2- Guarded ☐ 3- Fair ☑ 4- Good ☐ 5- Excellent ☐ 6- Other (Specify):

21. Treatments and Orders for Disciplines:
Frequency
PT Frequency: 3W2, 2W1 Starting Week of 02/24/2026,SN Frequency: 2W1, 1W2, 1EOW6,SN PRN Visits/Hours: 3 (Surgical complications)

Initial Order received:
Yes

Vitals Normal Range
BP Systolic: 90 - 160
BP Diastolic: 56 - 100
Pulse: 56 - 100

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Respiratory: 12 - 28
Temperature: 96.5 - 100.5
O2 Saturation: 90 - 100

Vitals Physician Alert Range

Temperature: < 96.5 or > 100.5
Pulse: < 56 or > 100
Respiratory: < 12 or > 28
BP Systolic: < 90 or > 160
BP Diastolic: < 56 or > 100
O2 Saturation: < 90
Fasting Blood Sugar: < 70 or > 250
Random Blood Sugar: < 70 or > 300
Weight: < 140 or > 150
Pain: > 7

SN

BEST PRACTICE FALL PREVENTION:

• Clinician will perform fall assessment and instruct patient /caregiver on fall prevention.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION:

• Clinician to perform drug regimen review/assessment to assess for potential clinically significant medications issues.

• Clinician to report potentially clinically significant medications issues to physician within 1 calendar day.

• Clinician to assess/instruct on high risk and other medications to include effects, potential adverse effects, interactions and duplicate therapy, dose route and frequency.

BEST PRACTICE MONITOR AND MITIGATE PAIN:

• Clinician will instruct patient /caregiver on methods to mitigate pain to include medications and non-pharmacological interventions to decrease pain.

BEST PRACTICE PRESSURE ULCER PREVENTION:

• Clinician will perform teaching and training to patient/caregiver for pressure ulcer prevention.

GENERAL: BEST PRACTICE DIABETES:

• Clinician to assess/perform monitoring for the presence of skin lesions on the lower extremities and perform patient/caregiver education on diabetic foot care.

SURGICAL WOUND:

• Clinician to assess surgical wound, instruct/perform wound care as follows: dressing is not to be removed by patient or home health. Assess for redness and increased drainage with percentage of strike through observable. Notify MD of any changes.Home health to provide wound care supplies as ordered. Clinician to discontinue wound care and wound care supplies when wound is healed.

HOSPITALIZATION RISK FACTORS:

• Clinician will instruct patient/caregiver on ways to prevent unintentional weight loss and poor nutrition (high calorie foods, recording (wt/diet), etc

• Clinician will instruct patient and/or caregivers on polypharmacy, duplicate medications, adverse effects, et and compliance/understanding of medication regimen.

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- Clinician to instruct patient and/or caregivers on ways to avoid exhaustion and fatigue to include adequate sleep, nutrition, exercise, etc.
- Clinician will instruct patient and/or caregivers on fall prevention to include clear pathways, adequate lighting, assistive devices, slow change in position, etc.

CARDIOVASCULAR - HTN:

- SN to instruct patient/caregiver that uncontrolled HTN can lead to heart attack, stroke, heart failure, kidney complications, vision loss, metabolic syndrome and vascular dementia. SN to instruct pt/cg that HTN is known as the silent killer and symptoms may not be obvious.
- SN to instruct the patient/caregiver on the following symptoms could be signs of a heart attack: chest discomfort, discomfort in one or both arms, back, neck, jaw, stomach, shortness of breath, cold sweat, nausea, or dizziness. Instruct patient/caregiver on signs and symptoms that necessitate calling 911.
- SN to instruct patient/caregiver on Heart Healthy diet.

PT

PHYSICAL THERAPY:

- Establish Home Exercise Program and Activity Program
- Home safety evaluation and fall prevention/teach fall risk reduction strategies
- Therapeutic Exercises: PROM/AAROM/AROM
- Therapeutic Exercises: Balance, coordination, and/or agility training
- Gait Training: Asst device (specify) _____RW___
- Transfer Training: Sit <-> Stand

Assessment Summary

Focus of care: aftercare ortho R TKA
Admission of a 68-year-old female with a history of OA, hyperlipidemia and HTN. Patient had a right total knee replacement and a home health referral was made.
The patient currently lives in a single story home with her spouse .She is a fall risk due to impaired mobility and generalized weakness. Durable Medical Equipment (DME) includes a walker and cane. The patient is homebound due to significant weakness and impaired transfers, making it a taxing effort to leave the home. She requires assistance with activities of daily living (ADLs) and utilizes a walker for mobility.
The patient is admitted to Be Healthy Home Health for skilled nursing (SN) assessments, medication management, aftercare TKA and HTN education. SN instructed the patient and caregiver on proper medication administration, including Using weekly pill packs. The skilled nurse also to teach on Fall precautions and skin protection for the prevention of breakdown.
Physical Therapy (PT) to evaluate and assist with mobility and transfer techniques, focusing on improving strength and reducing fall risk.
The patient and caregiver stated their goals for home health are to feel safe at home, manage medications properly, and improve mobility for increased independence. SN's focus of care is on managing aftercare TKA,ensuring proper medication compliance, and preventing complications related to recent health changes. Patient has follow up appointment with surgeon on 3/5/2026.

Frequency may be completed with both face to face visits and/or telehealth (if appropriate to meet patient treatment needs).
All qualified disciplines may do non-billable visits to supervise staff and collect oasis data as needed.

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Within 5 days of the end of episode, a qualified professional may have a PRN visit to complete a comprehensive/OASIS assessment to establish the need for continuing home health services or discharge.
May accept orders from all physicians caring for the patient.
Any qualified disciplines may perform visit for resumption of care when patient discharged home from inpatient facility.
May resume all care prior to the hospitalization.
PAM Health at Home may discharge patient if remains in facility when certification period expires.
All skilled disciplines may check oxygen saturation prn sob.

I certify that the patient is confined to his/her home and needs intermittent skilled nursing, physical therapy, occupational therapy and/or speech therapy for the next 60 days.
I attest to the following:
1. Rodriguez, Rosemary C is under my care.
2. I have received the following patient records from PAM Health at Home:
a. Patient Summary Form
b. Hospital/LTC stay information (if applicable)
c. Home Health Plan of Care
d. And the signature below is my signature
e. These patient records were considered in advance of my certification of the patient's eligibility for home health services under Medicare.
I certify that this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy or continues to need occupational therapy for the next 60 days. This patient is under my care, and I have authorized the services on this plan of care and I or another physician will periodically review this plan. I attest that a valid face-to-face encounter has occurred on 2.11.2026 BY JASON THOMPSON, MD and it is related to the primary reason the patient requires home health services. PAM HEALTH AT HOME, HOME HEALTH AGENCY, MAY PERFORM SOC ON 02.24.2026.

Risk of Hospitalization

Risks: History of falls (2 or more falls - or any fall with an injury - in the past 12 months); Decline in mental, emotional, or behavioral status in the past 3 months; Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months; Currently taking 5 or more medications; Currently reports exhaustion;
Interventions: Referrals - SN,PT; Medication Management - Reconciliation, Assess patients: Knowledge,Ability,Resources & Adherence, Education; Patient/Family Education; Phone Monitoring; Front-loading Visits; Fall Prevention Program; Individualized Patient Emergency Care Plan; Care Coordination (Physicians, Hospitals, Nursing homes…); Tele monitoring
Risk Factors: Discharged from Hospital or Skilled Nursing Facility (M1000); Help with managing Medications needed (M2020); More than 2 Secondary Diagnoses (M1022 & M1024); Confusion (M1710); Dyspnea (M1400) ; ADL Assistance Needed (M2102 & M2110)

Psychosocial Assessment

No problem found

Cognitive Status

Cognitive status: No behaviors demonstrated

Advance Directive

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Patient/Caregiver reports no Advanced Directives in place

Homebound Status

Patient Homebound Reasons
- Is the Patient Homebound: Yes
- Homebound Reasons: Need assistance for all activities; Require assistance to ambulate; Unable to safely leave home unassisted; Severe SOB, SOB upon exertion; Dependent upon adaptive device(s);
- Patient Chronic and Stable: Yes
- Patient leaving the home require Considerable and Taxing effort : Yes
- Comments:

22. Rehabilitation Potential/Discharge Plans/Goals :

Rehab Potential

Good to achieve stated goals with skilled interventions and patient's compliance with plan of care.

Discharge Plan

Discharge when medical condition is stable and patient is no longer in need of skilled services

Goals

SN

BEST PRACTICE FALL PREVENTION: - LTG
- LTG-Patient/caregiver will verbalize understanding of interventions to prevent falls by end of episode.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION: - LTG
- LTG-Patient /caregiver will be able to name s/s of adverse side effects to report to MD by end of episode.
- LTG-Patient /caregiver will be able to verbalize/demonstrate correct dose, route, and frequency of all medications and will be able to recall adverse effects to report to MD by end of episode.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION: - STG
- STG-Patient /caregiver will be able to name s/s of adverse side effects by week 3.

BEST PRACTICE MONITOR AND MITIGATE PAIN: - LTG
- LTG-Patient/caregiver will demonstrate decreased pain with ambulation as evidenced by increased activity tolerance by end of episode.

BEST PRACTICE PRESSURE ULCER PREVENTION: - LTG
- LTG-Patient/Caregiver will demonstrate/verbalize understanding of methods to prevent pressure ulcers by end of episode.

GENERAL: BEST PRACTICE DIABETES: - STG
- STG-Patient/Caregiver will demonstrate ability to perform proper diabetic foot care as evidenced by Keeping a daily log by 6 weeks.

SURGICAL WOUND: - STG
- STG-Patient/Caregiver will be able to identify s/s of infection of surgical wound by week 2.

HOSPITALIZATION RISK FACTORS: - LTG
- Patient and/or caregivers will verbalize understanding of ways to mitigate risk of return to the hospital and unplanned ER use.

CARDIOVASCULAR - HTN: - LTG

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- Patient/caregiver will be able to verbalize ways to manage HTN by end of episode.

PT

HEP: - STG

- STG-Patient and/or caregiver will complete Home Exercise Program at Indep, within __2___weeks.

MOBILITY/GAIT: - LTG

- LTG-Patient will demonstrate improved trasnfer ability , from min assist, to mod indep s loss of balance, to promote safwe transfers, within _4___ weeks. or at end of episode.
- LTG-Patient will demonstrate improved mobility as evidenced by a categorical change with a decreased TUG time from _120__ seconds to _30___ seconds(or less s loss of balance), with RW, in order to promote safe functional abiliteis and mobility, within __4__weeks. or at end of episode.
- LTG-Patient will demonstrate improved mobility as evidenced by the increased ability to ambulate with RW for 150 feet(from 20 feet at min assist) at Mod indep s loss of balance, in order to safely ambulate in residence and to vehicle, within __4__weeks. or at end of episode.
- LTG-The pt will demonstrate improved sx knee rom (from 0 -75 degrees) to within surgeon's goals within his timeframe, within __4__weeks. or at end of episode.

23. Clinician's Signature and Date of Verbal SOC Where Applicable: (Electronically Signed)
Verbal SOC Date: 02/24/2026
Signed By: Stewart, Amber - RN on
02/24/2026 12:07 PM, Cosigned by
Strait, Donna - RN

25. Date HHA Received Signed POT:

24. Physician's Name and Address
JASON THOMPSON MD
601 TEXAN TRL STE 300
Corpus Christi, TX 78411,NPI: 1326465576, License#: S4573
,Phone: (361) 854-0811, Fax: (361) 806-5040

26. I certify/ recertify that this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy or continues to need occupational therapy. This patient is under my care, and I have authorized the services on this plan of care and I or another physician will periodically review this plan. I attest that a valid face-to-face encounter occurred (or will occur) within timeframe requirements and it is related to the primary reason the patient requires home health service

27. Attending Physician's Signature and Date Signed:

Electronically signed by JASON THOMPSON, MD on 03/03/2026 at 08:56:26 am

28. Anyone who misrepresents,falsifies, or conceals essential information required for payment of Federal funds may be subject to fine, imprisonment, or civil penality under applicable Federal laws.